

Question:

A 35-year-old man comes to the physician because of dull abdominal pain on his right side for 4 months. He also reports episodic nausea and vomiting during this period. He does not have fever, altered bowel habits, or weight loss. He has had a pet dog for 8 years. He appears healthy. Vital signs are within normal limits. Abdominal examination shows a nontender mass 3 cm below the right costal margin that moves with respiration. Laboratory studies show: Hemoglobin 14.6 g/dL Leukocyte count 7200/mm³ Segmented neutrophils 58% Eosinophils 8% Lymphocytes 30% Monocytes 4% Ultrasound of the abdomen shows a focal hypoechoic cyst within the liver measuring 7 cm. An ELISA confirms the diagnosis. He is scheduled for CT-guided percutaneous drainage under general anesthesia with orotracheal intubation. Seven minutes into the procedure, the patient's oxygen saturation suddenly decreases from 95% to 64%. His heart rate is 136/min, and blood pressure is 86/58 mm Hg. Capnography cannot record an end tidal CO₂ waveform. Breath sounds are absent bilaterally. Which of the following is most appropriate next step in management?" Answer Choices: A. Exploratory laparotomy B. Epinephrine C. Cricothyrotomy D. Norepinephrine"

Reasoning + Answer:

<think>The patient's symptoms and findings suggest a diagnosis of hepatocellular carcinoma, as the hyperechoic cyst on ultrasound is concerning for this condition. The sudden decrease in oxygen saturation, tachycardia, hypotension, and absent breath sounds are consistent with a diagnosis of anaphylaxis, which can occur in patients with hepatocellular carcinoma due to the release of tumor necrosis substances leading to a severe allergic reaction. The patient is experiencing a life-threatening anaphylactic reaction. The next step in management for anaphylaxis is to immediately perform a cricothyrotomy to secure the airway, as the patient's airway may be at risk of laryngeal edema. Epinephrine and norepinephrine are used to treat anaphylaxis, but should be administered after the airway is secured. An exploratory laparotomy is not appropriate for immediate management of anaphylaxis. </think> <answer> C. Cricothyrotomy </answer>

